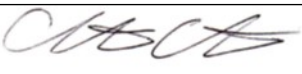


Guideline Title: Gum Elastic Bougie

Guideline Number: GUID-3203-PR011

Issue date: 09/09/2014	Replaces Dept. Policy: End-Tidal CO2 Detector
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To assist with placing an oral endotracheal tube during difficult intubations or cricothyrotomy
- II. **DEFINITIONS:**
 - a. ETT – Endotracheal tube
- III. **DEPARTMENT GUIDELINE:**
 - a. **INDICATIONS:**
 - i. Should be considered/available in all intubations.
 - ii. Most useful when orotracheal intubation is considered difficult due to unfavorable anatomy or known or suspected neck trauma/limiting neck mobility
 - iii. Adult Bougie: ETT size ≥ 6.0
 - iv. Pediatric Bougie: ETT size < 6.0
 - b. **CONTRAINDICATIONS:**
 - i. Should not be used for nasal intubations
 - c. **PROCEDURE:**
 - i. Assess airway and need for a bougie.
 - ii. Prepare equipment needed for endotracheal intubation as indicated per procedure *GUID3203-PR020 – Endotracheal Intubation*.
 - iii. Begin intubation procedure as indicated in *GUID3203-PR020 – Endotracheal Intubation*.
 - iv. Perform laryngoscopy and when anatomy is identified, pass the bougie into the trachea or anteriorly toward the presumed opening of the trachea. Tracheal placement of the bougie is noted by a clicking feel/sound as the flexed tip passes over the tracheal rings.
 - v. Advance bougie until the device is well inside the tracheal lumen (There should be ‘hold up’ at 30-40 cm depth. If this does not occur the bougie is likely to be in the esophagus. A typical bougie is 60 cm so 1/3 to 1/2 of bougie should remain out of the patients mouth when “hold up” is achieved adults.)
 - vi. Thread the proximal end of the bougie through the distal end of a lubricated ETT.

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- vii. Advance the tracheal tube over the bougie until the bougie exits the proximal opening in the tracheal tube. If possible, continue laryngoscopy when advancing ETT over bougie, to assist ready passage of the endotracheal tube.
- viii. If resistance is encountered at the vocal cords, withdraw the tracheal tube 1-2 cm, rotate it 90 degrees counterclockwise and then re-advance the tracheal tube.
- ix. When the tracheal tube is clearly in the trachea, remove the bougie and inflate the cuff.
- x. Confirm proper ETT placement by calculating 3 x the ETT size. Ventilate the patient with 100% oxygen. Secure the ETT.

IV. DOCUMENTATION:

- a. EMS charts

V. REFERENCES: